



NMHS PROCEDURE

Voluntary Assisted Dying

Scope (Staff)	All clinical and non-clinical staff
Scope (Area)	All NMHS sites and services
Summary	This Procedure facilitates the NMHS Voluntary Assisted Dying Policy and is in accordance with the <i>Voluntary Assisted Dying Act 2019 (the Act)</i>. This Procedure has been reviewed and updated to include the following revisions: • expansion and clarification of definitions • clarification of information regarding use of language services; • clarification of medical staff obligations under the Act; • clarification of the admission referrals and pathways for inpatient administration of VAD Substance; • clarification of requirements for supply of the VAD substance.
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1. Aim

The North Metropolitan Health Service (NMHS) Voluntary Assisted Dying Procedure facilitates the implementation of the [NMHS Voluntary Assisted Dying Policy](#).

This Procedure is to be read alongside the [Voluntary Assisted Dying Act 2019 \(the Act\)](#) and in conjunction with the [Department of Health Managing Voluntary Assisted Dying Policy \(MP 0154/21\)](#) and the [Western Australian Voluntary Assisted Dying Guidelines](#) (WA VAD Guidelines) as well as the [NMHS Voluntary Assisted Dying Policy](#) (NMHS VAD Policy). In addition, several Department of Health (DoH) approved resources are referenced in this Procedure to assist practitioners and others.

This Procedure provides information for NMHS staff regarding the processes and management of VAD at NMHS.

The information contained in this Procedure is not intended to be exhaustive and is not a substitute for the Act. Health professionals who are considering participating in or supporting VAD in WA must familiarise themselves with the Act.

2. Key VAD Roles and Contacts

2.1 NMHS VAD Coordinator

The role of the [NMHS VAD Coordinator](#) includes the following:

- provide information, support, advice and assistance to patients, families and staff across NMHS in relation to VAD and associated processes;
- liaise between the Statewide Care Navigator Service, Statewide Pharmacy Service, participating VAD practitioners, NMHS sites and patients under or referred to their care;
- provide support and assist preparing staff for VAD activity on NMHS sites; and
- liaise with the executive teams as appropriate to facilitate processes to address patient and practitioner needs regarding VAD.

[NMHS VAD Coordinator](#) can be contacted by email or mobile during normal business hours. Out-of-hours messages will be followed up as soon as practicable on the next business day.

- Email: NMHS.VAD@health.wa.gov.au
- Phone: 0479 177 780

2.2 VAD Practitioners

The Act allows Medical Practitioners and Nurse Practitioners who meet the eligibility requirements, and who have successfully completed the WA VAD Approved Training to participate in the voluntary assisted dying process as a VAD Practitioner (see [section 16. Definitions](#)).

Medical practitioners who are eligible and have successfully completed the WA VAD Approved Training can actively undertake the roles of:

- Coordinating Practitioner
- Consulting Practitioner
- Administering Practitioner

Nurse Practitioners who are eligible and have successfully completed the WA VAD Approved Training can undertake the role of Administering Practitioner.

Refer to [WA VAD Guidelines, section 4.2](#) for eligibility criteria.

NMHS supports VAD practitioners, both internal and external to NMHS, seeking access to NMHS sites to support patients with the VAD process. All VAD practitioners (both internal and external to NMHS) must be credentialed to include VAD in their scope of clinical practice by the relevant Credentialing Committee. Practitioners external to NMHS will need to be credentialed on a case-by-case basis at the discretion of the relevant Credentialing Committee before performing VAD related roles at NMHS sites.

The [NMHS VAD Coordinator](#) can be contacted to facilitate timely, out-of-session credentialing to avoid delaying the VAD process.

2.3 Statewide Care Navigator Service (SWCNS)

SWCNS has been established to provide information, support, advice and assistance to anyone involved in VAD in WA. It can:

- provide general information about VAD;
- provide specific information about VAD in WA;
- help to locate a medical practitioner or nurse practitioner who is willing and eligible to participate in VAD;
- assist people to access regional support packages; and
- link people to other helpful resources.

SWCNS can be contacted by email or phone during standard work hours (8:30am – 5:00 pm).

- Email: VADcarenavigator@health.wa.gov.au
- Phone: (08) 9431 2755

2.4 Statewide Pharmacy Service (SWPS)

The SWPS has been established to ensure that the VAD substance is provided in a safe, equitable and patient-centred manner. It provides the following functions:

- liaise with medical practitioners and nurse practitioners who are supporting someone through the VAD process;
- supply the VAD substance;
- provide information, education and support to a medical practitioner, nurse practitioner, or person accessing VAD;
- liaise with Authorised Disposers regarding disposal of any unused or remaining VAD substance.

SWPS can be contacted by email and phone during standard work hours (8:30 am – 5:00 pm).

- Email: StatewidePharmacy@health.wa.gov.au
- Phone: (08) 6383 3088

3. Cultural Considerations

3.1 Aboriginal People

Aboriginal people may have social, cultural and emotional concerns regarding VAD. In some Aboriginal languages there is no equivalent translation for the concept of VAD. Aboriginal concepts of individual and community wellbeing are strongly linked to culture and spirit. This holistic view of wellbeing incorporates cyclical concepts of life, death and spiritual dreaming. Discussions around end-of-life care and choices with Aboriginal patients should identify, recognise and respect these key components of wellbeing, while not presuming to exclude them from the option of VAD.

Interpreting assistance from a carer, family member or friend of the patient **is not permitted** under the Act. Language services can be used to facilitate communication with the patient in relation to VAD. Refer to [Section 3.3 - Use of Language Services](#).

Refer to DoH [Information about supporting Aboriginal people](#) for further information.

The [NMHS VAD Coordinator](#) or the [WA VAD Statewide Care Navigator Service \(SWCNS\)](#) can assist with linkage to Aboriginal Liaison Officers and healthcare workers who support Aboriginal people.

3.2 People of Culturally and Linguistically Diverse (CaLD) Background

Similarly, people of a CaLD background may have social, cultural and emotional concerns regarding VAD. For patients who intend to access VAD, it is important that communication throughout each stage of the process occurs in a culturally aware and sensitive manner. Language services can be used to facilitate communication with the patient in relation to VAD. Interpreting assistance from a carer, family member or friend of the patient **is not permitted** under the Act. Refer to [Section 3.3 Use of Language Services](#).

3.3 Use of Language Services

Under the Act, only an interpreter who has been accredited by a body approved by the Director General for Health can be used to facilitate communication in relation to VAD. Refer to DOH [Information for interpreters](#) for further information.

Use of Language Services during the VAD process should comply with the [WA Health Language Services Policy \(MP 0051/17\)](#), and documented in the patient's healthcare record.

It is important to discuss the intended use of the interpreter service at the time of booking to ensure the interpreter assigned does not have a conscientious objection to involvement in the VAD process. Interpreters will be offered the same support and choice of level of involvement as NMHS staff. It is also important to be aware of the Commonwealth *Criminal Code Act 1995* constraints relating to the use of telephone interpreting services in relation to VAD (Refer to [NMHS VAD Policy](#) Section 12. The Legal Context of VAD for Practitioners). Additional assistance can be provided by the [NMHS VAD Coordinator](#) or the [SWCNS](#).

4. Who can initiate a discussion about VAD?

A patient may raise the topic of VAD with anyone involved in their care.

A [medical practitioner](#) or [nurse practitioner](#) can initiate a discussion about VAD with a patient if, at the same time, they also inform the patient of the palliative care and treatment options available to the person and the likely outcomes of that care and treatment.

Raising VAD outside of this constitutes unprofessional conduct within the meaning of the *Health Practitioner Regulation National Law (Western Australia) Act 2010*.

If the patient raises it first, healthcare workers can respond to questions about VAD and provide information, if they are comfortable doing so. In addition, a person seeking information can be directed to the WA DoH VAD website. Otherwise, patients can be provided with the contact details of the [NMHS VAD Coordinator](#) or the [SWCNS](#) who can assist them further.

A patient raising the topic of VAD presents an opportunity for health professionals involved in their care to have a meaningful discussion about their care and support needs, symptom management, palliative care options, support for their family and their priorities as they approach the end of their life.

The principles underpinning the Act include that a patient should be encouraged to openly discuss death and dying, and to be supported in conversations about treatment and care preferences. Linkage or referral to other services may form part of this discussion with the patient.

A discussion about VAD, whether initiated by the patient or by a medical practitioner or nurse practitioner, **does not** begin the VAD process.

5. The VAD Process

There are ten (10) steps in the VAD process (see **Figure 1**). Steps 1 to 6 involve the request and assessment process, while steps 7 to 10 relate to the administration process. This Procedure provides relevant information and NMHS approaches in each step. For detailed information on the VAD process, staff are recommended to refer to [the Act](#), the [WA VAD Guidelines](#) and the [WA Health VAD website](#). [Appendix 1](#) provides details on staff roles and responsibilities during the VAD process.

Figure 1. The voluntary assisted dying process



5.1 First Request

5.1.1 What is a First Request

A First Request for VAD **must** be:

- clear and unambiguous;
- made to a medical practitioner;
- made during a medical consultation; and
- made in person or via videoconference.

A request for VAD **must meet all the criteria** listed above to be considered a First Request.

Regardless of whether the medical practitioner perceives the patient may be eligible or ineligible to access VAD, the medical practitioner must complete steps 1) to 4) as outlined in Box 1, section 5.1.3 of this document.

Patient eligibility to access VAD will be assessed at a later stage (after the First Request) by the Coordinating Practitioner and can only be determined by a VAD practitioner.

5.1.2 What is not considered a First Request

Some patients nearing the end of life may make Desire to Die (DTD) and Desire to Hasten Death (DHD) statements. These statements should **not** be interpreted as requests for VAD. Instead, these statements are an expression of suffering and should be explored with openness and curiosity with the aim of determining the underlying concerns and how they may be best addressed. Discussions with the patient should include exploration of other supports available to them and referral to services such as social work, psychology, counselling and other supportive services as appropriate. Some examples of what would not be considered as a First Request include:

- A person seeking more information or expressing curiosity about VAD;
- A person making a request for VAD to a health professional who is not a medical practitioner; or
- A person making a request for VAD to a medical practitioner but not during a medical consultation.
- A person's carer, family member, medical treatment decision-maker or friend requesting VAD on the person's behalf.

Table 1 Responding to patient statements

Responding to patient statements				
The following statements made by a patient (listed below) can be considered:	Desire to Die (DTD) statement	Desire to Hasten Death (DHS) Statement	Insufficient request for VAD	Clear and unambiguous request for VAD
	<i>Acknowledge, Ask, Assess</i>	<i>Acknowledge, Ask, Assess</i>	<i>Acknowledge, Ask, Assess</i>	<i>Acknowledge, Ask, Assess</i> <i>Continue conversation about VAD if appropriate</i>
"Can you give me all the options?"			✓	
"I am tired of life and I just want to die."	✓		✓	
"I've had enough of all of this. I just want to get it over with"		✓	✓	
"Isn't there something you can do to put an end to this?"		✓	✓	
"If animals can be put down when they are suffering. Why can't I?"			✓	
"I would like you to assist me to die"	✓	✓		✓
"Can you help me die?"	✓	✓		✓
"How do I get that pill they say you can get to end it all that I can take when it all gets too much?"	✓	✓		✓
For statements that ARE NOT a sufficient request for VAD, continue to explore. <i>Acknowledge, Ask, Assess</i>				
For statements that ARE clear and unambiguous requests for VAD, discuss VAD. <i>Acknowledge, Ask, Assess</i>				

Table 1 outlines examples of what patients might say and how health practitioners should respond. If appropriate to your role, acknowledge the patient's statement, ask why the patient has made this statement at this time and what the patient's concerns are. Assess the patient's physical, psychological, existential and social issues, and offer relevant supports.

The treating team should consider the need to initiate advance care planning discussions, review Goals of Patient Care/Care Plan and refer to other specialties if needed, including but not limited to Palliative Care.

Conversations should be appropriate to the role of the staff member.

5.1.3 Responding to a First Request

Only medical practitioners can receive a First Request. If a patient makes a First Request to any other health professional, they should be informed that they should make their request to a medical practitioner and also be referred to the [NMHS VAD Coordinator](#) as soon as practicable to guide them with accessing an eligible and available medical practitioner.

- A medical practitioner is not obliged to *accept* a First Request. Deciding whether to accept or refuse a First Request is a personal choice. Several aspects should be considered in making this decision including:
 - their willingness to be involved;
 - their ability to perform the necessary duties; and
 - their eligibility to fulfil a role under the Act.

Box 1 Mandatory steps required under the Act

If a patient has made a First Request, the medical practitioner must complete below steps, regardless of whether accepting or refusing the First Request:

- 1) Accept or refuse the First Request and advise the patient of their decision;
- 2) Provide the patient with the [Approved information for a person making a First Request for voluntary assisted dying](#) (available from the WA DOH VAD website);
- 3) Record in the patient's healthcare record the following information:
 - a First Request has been made;
 - the decision to accept or refuse;
 - if refused, reason for refusal; and
 - if they have provided the patient with the approved information;
- 4) Complete and submit the *First Request Form* to the VAD Board within two (2) business days of receiving the request via the VAD – Information Management System ([VAD-IMS](#)) or via fax. The form is available on the [VAD-IMS](#) website and log-in is not required.

Failure to complete any of these four (4) steps is a breach of the Act.

Submission of the First Request Form to VAD-IMS does NOT trigger patient follow-up. The [NMHS VAD Coordinator](#) must be contacted for patient follow-up regardless of the decision to accept or refuse the First Request

- Junior medical staff (interns, resident medical officers, and registrars less than PGY10) are not eligible to fulfil a role under the Act and must refuse the First Request and inform their consultant. They must complete steps 1) to 4) as described in Box 1 below.
- With the patient's consent, the junior medical officer should advise the consultant of the treating team of the patient's request for VAD and inform the [NMHS VAD Coordinator](#) as soon as practicable.
- If a medical practitioner refuses the First Request on the basis of conscientious objection, they must inform the patient **immediately** and complete the steps 1) to 4) as described above, and notify the [NMHS VAD Coordinator](#) as soon as practicable.

- If a medical practitioner wishes to have time to consider the request, they must inform the patient of their decision within two (2) business days (not including the day the First Request was made), and complete the steps 1) to 4) as described above, and notify the [NMHS VAD Coordinator](#) as soon as practicable.
- A medical record form (VAD – First Request Form) is available to facilitate documentation of First Request in the patient's healthcare record (see [Section 14 - Documentation](#)).

Refer to [VAD Conversation Guide](#) located on the [NMHS VAD Hub](#) for guidance on conversations with the patient in regards to VAD.

5.1.4 Care of the patient after First Request

A patient raising the topic of VAD provides an opportunity for their treating team and consulting team/s to have meaningful discussion with the patient about their care needs and symptom management. The patient's management plan, including the *Goals of Patient Care Form*, must be reviewed and updated. Someone whose First Request to VAD is refused may require some sensitivity and an explanation of the refusal. In these instances, additional support and offers of information should be considered. The [NMHS VAD Coordinator](#) will be able to assist further and should be contacted as soon as practicable. The patient can also contact the [SWCNS](#) for guidance.

If a medical practitioner accepts the First Request, they become the patient's Coordinating Practitioner. They do not need to have completed the WA VAD Approved Training to accept the First Request, but they must complete it before beginning the First Assessment.

5.2 First Assessment

Once the medical practitioner accepts the First Request, they become the Coordinating Practitioner for the patient and will coordinate the VAD process for them. The Coordinating Practitioner must formally assess the patient's eligibility to access VAD against the eligibility criteria which includes:

- assessing the person's demographic eligibility
- assessing whether the person's disease, illness or medical condition meets the eligibility criteria
- assessing the person's decision-making capacity

A record that a First Assessment by a VAD Practitioner has been conducted on a NMHS site should be entered in the patient's healthcare record in accordance with the the Documentation requirements ([section 14](#) of this Procedure). It is acceptable to document relevant clinical information when describing the assessment , as long as it informs how a determination of the eligibility was reached.

This process is covered in detail in the mandatory WA VAD Approved Training and in the [WA VAD Guidelines](#). NMHS sites support a First Assessment by a Coordinating Practitioner who can be NMHS staff or non-NMHS staff member, once appropriately credentialed as per [Section 2.2](#).

Where the Coordinating Practitioner is unable to determine the patient's eligibility to access VAD they **must** refer the patient to a registered medical practitioner or other professional with the appropriate skills and training to provide a determination in relation to the matter. This should follow the usual referral process at each NMHS site.

- Discussions between the referring practitioner and the receiving practitioner must occur prior to the referral being made.

- The registered medical practitioner or professional receiving the referral for determination in eligibility is fulfilling a role under the Act, and as such is subject to the same confidentiality requirements as the Coordinating Practitioner and Consulting Practitioner.
- The registered medical practitioner or professional who accepts the referral **must not** be a family member of the patient or be a beneficiary under the Will of the patient and **does not** need to have completed the WA VAD Approved Training to accept and complete the supplementary assessment.
- The [NMHS VAD Coordinator](#) or the [SWCNS](#) can assist with referrals if required.

NMHS supports Coordinating Practitioners to make specialist referrals to NMHS staff to assist in the VAD eligibility assessment of a patient following a First Request and supports the acceptance of referrals by NMHS specialist practitioners for the purpose of VAD eligibility assessment.

5.3 Consulting Assessment

If the patient has been assessed as eligible for VAD by the Coordinating Practitioner, a Consulting Assessment will be undertaken by a Consulting Practitioner.

NMHS sites support a Consulting Assessment by a Consulting Practitioner who can be NMHS staff or non-NMHS staff member.

A record that an assessment by a VAD participating practitioner has been conducted on an NMHS site should be entered in the patient's healthcare record in accordance with the Documentation requirements ([Section 14](#) of this Procedure). It is acceptable to document relevant clinical information when describing the assessment, as long as it informs how a determination of the eligibility was reached.

5.4 Written Declaration

If the patient has been assessed as eligible for VAD by the Coordinating Practitioner and the Consulting Practitioner, the patient may then make a **Written Declaration** of their request for VAD in the presence of two (2) eligible witnesses. To be eligible to witness the signing of the Written Declaration, the two witnesses must:

- have reached 18 years of age;
- not be a beneficiary under the Will of the patient or may otherwise benefit financially or in any other material way from the death of the patient;
- not be a family member of the person; and
- not be the Coordinating Practitioner or Consulting Practitioner for the patient.

The purpose of acting as a witness for a Written Declaration is to consider if the patient appears to freely and voluntarily sign the Written Declaration, and if so, to certify this in writing.

NMHS recommends that NMHS staff do **not** act as witnesses for a Written Declaration, however, NMHS staff *may* act as witnesses should they choose to do so. In addition to meeting the above requirements to be a witness, the following requirements must also be met:

- asked by the patient to be a witness to the Written Declaration; and
- comfortable and willing to do so.

It is important to remember that by acting as a witness, the witness is signing a legal document and may be called upon to give evidence in a court or tribunal at a later date if the patient's Written Declaration is called into question. This should be considered by NMHS staff when deciding whether they are willing to be a witness. The [NMHS VAD Coordinator](#) can be contacted to provide further guidance if required.

5.5 Final Request

Once the Written Declaration is completed, the patient may then make a Final Request for VAD to the Coordinating Practitioner.

There is a designated minimum period of **nine days** required between the First Request and the Final Request. The designated time period begins on the day the person makes the First Request. However, a shorter period is allowed by the Act in specific circumstances as determined by the Coordinating Practitioner and Consulting Practitioner if the patient is likely to die or lose decision making capacity before the end of the designated period.

If the Final Request has been received by a VAD Practitioner on a NMHS site, a notation should be entered in the patient's medical record in accordance with the documentation requirements (Section 14 of this Procedure).

5.6 Final Review

Once they have received a Final Request, the Coordinating Practitioner must complete a Final Review. This process is covered in detail in the mandatory WA VAD Approved Training and in the [WA VAD Guidelines](#).

5.7 Administration Decision

The patient should decide, in consultation with their Coordinating Practitioner, whether they would self-administer the VAD substance or have an Administering Practitioner administer it to them.

5.7.1 Self-administration

If a patient makes a self-administration decision, they must appoint a Contact Person before the Coordinating Practitioner can prescribe the VAD substance. Refer to [DoH Choosing the Contact Person information sheet](#) for further information.

The Administration Decision must be communicated in person to the Coordinating Practitioner in order to comply with the Commonwealth Criminal Code Act. The Administration Decision must be recorded in the patient's healthcare record.

Patients are encouraged to choose a Contact Person who is **not** staff of NMHS.

NMHS staff **should not** be a Contact Person for a patient accessing VAD unless the patient is a family member/relative, next of kin or close friend. In exceptional circumstances, where all other alternatives are exhausted, NMHS staff *may* act as a Contact Person for inpatient administration. In both situations, the staff member must have a full understanding of the role and responsibilities of a Contact Person under the Act before agreeing to take on the role (refer to [DoH Being the Contact Person information sheet](#)). The [NMHS VAD Coordinator](#) can be contacted to provide further guidance if required.

5.7.2 Practitioner Administration

A practitioner administration decision can be made if the Coordinating Practitioner considers that self-administration of the VAD substance is not appropriate for the patient due to one or more of the following reasons:

- the patient is unable to self-administer the substance;

- the patient has concerns about self-administering the substance; and/or
- the self-administering method is unsuitable for the patient.

6. Inpatient admission process

A patient can be admitted into the acute hospital setting via different pathways, each with a specific process to be followed. These pathways include:

- Direct admission from the community for the purposes of VAD administration
- Direct admission from another inpatient hospital setting for the purpose of VAD administration
- Admission via the emergency department (ED) seeking admission for the purpose of VAD administration
- Admission via the emergency department for reasons other than VAD administration (i.e. management of an acute clinical issue)

6.1 Direct admission from the community or another inpatient hospital setting for the purposes of VAD administration

Referrals for elective admissions for the purpose of VAD Administration at NMHS may come from:

- The patient directly
- The [SWCNS](#);
- Medical Practitioners from inpatient or outpatient settings at NMHS;
- Faith-based or other organisations within the NMHS catchment
- General Practitioners (GPs) within the NMHS catchment.

Referrals from the above sources for elective admissions for the purpose of VAD Administration at NMHS should be coordinated through the [NMHS VAD Coordinator](#) and acceptance will be determined at site level and endorsed by the site Executives, following site-specific pathways.

Any patient seeking an elective admission for the purposes of VAD Administration at NMHS will be admitted under the care of either their known treating consultant, another identified speciality consultant, or under the Director of Clinical Services (or their delegate) once acceptance of care has been confirmed by the NMHS VAD Coordinator.

If a bed is not available at the preferred NMHS site, the [NMHS VAD Coordinator](#) may contact other sites or the [SWCNS](#) to facilitate alternative arrangements, in consultation with the Coordinating Practitioner.

Planning for VAD administration will generally be undertaken during usual business hours. Administration of the VAD substance will be discussed and planned with the patient and the Coordinating/Administering Practitioner.

6.2 Emergency Department presentation for the purpose of VAD administration

Patients should not be encouraged to present to ED seeking admission for VAD administration. However, if a patient does present requesting admission for the purpose of administering the VAD substance and a bed is available, the patient should be admitted to an appropriate ward. The patient should be referred to their known treating team/area of specialty for admission. The admission reason should be documented as the patient's principal underlying disease and staff should notify the [NMHS VAD Coordinator](#) of the patient's ED presentation and outcome.

If no bed is available, the patient should be given the opportunity to wait for an available suitable bed or be encouraged to go home if safe to do so and advised they will be followed up by [the NMHS VAD Coordinator](#) on the next business day.

6.3 ED presentation for reasons other than VAD administration

If the patient requires inpatient admission from the ED presentation, routine admission processes should be followed and inpatient management of the VAD kit must be adhered to (see [Section 9.3 - Storage of VAD substance at NMHS](#)).

Staff should notify the [NMHS VAD Coordinator](#) of the patient's ED presentation and outcome.

7. Inpatient requests for VAD administration

If an inpatient of NMHS requests VAD administration during an admission, the [NMHS VAD Coordinator](#) should be notified to ensure site-specific pathways are followed. A higher level of confidentiality of the patient and their participating practitioner(s) should be maintained and respected at all times. Information communicated during shift handover and ward rounds must Refer to [Section 14 – Documentation for further information](#)

8. Inpatient Management of the VAD Kit

Where a patient presents to the Emergency Department (ED) or directly to an inpatient ward for admission with the VAD Kit containing the VAD Substance, **the VAD Kit must remain with the patient at all times during their admission.**

The VAD Kit should already be locked with a PIN-coded padlock, to which **only** the patient and possibly their Contact Person have the code. The code should not be shared with hospital staff or be recorded in the patient's healthcare record. The appointed Contact Person's name and contact details must be documented in the medical record for ease of contact during the admission if required.

- Staff must apply a tamper-evident tag to the VAD Kit (Refer to [Section 9.3 - Storage of VAD substance at NMHS](#), **Figure 2**) immediately on presentation/admission and alert the shift coordinator. A visual inspection of the tag should be conducted at least once every shift, or at staff changeover, to ensure the tag is secure and intact and the tag number documented in a dedicated register (Refer to [Section 9.3 Storage of VAD substance at NMHS](#), **Figure 2**) by the nurse with a witness (who can be either medical or nursing staff). The [NMHS VAD Coordinator](#) or the site Senior Registered Nurse (SRN)/Senior Registered Midwife (SRM) can be contacted to arrange the delivery of the dedicated register.
- The VAD Kit should be secured to the patient's bed/trolley. Lockable cabinets are available at each NMHS site to facilitate secure storage of the VAD Box at patient's bedside where practicable, however, the use of these lockable cabinets is not mandatory. Refer to [section 9.3 Storage of VAD substance at NMHS](#). The [NMHS VAD Coordinator](#) can be contacted to arrange the access to the lockable cabinet or a suitable lock for attaching to the patient's bed/trolley. One key to either the cabinet or lock should be left with the patient and the other key attached to the controlled drug keys for the duration of the patient's stay in the clinical area. If the patient moves clinical areas, then the other key must be handed over to the holder of the controlled drug keys in the new clinical area.
- The dedicated register should be kept with the lockable cabinet (if used) or the VAD Kit **at all times** and returned to the NMHS VAD Coordinator or hospital pharmacy at the end of the patient's admission. **The key to the cabinet or lock is to be retrieved from the patient upon discharge and returned to the NMHS VAD coordinator.**

- If the tamper-evident tag is found to be broken, a new tag should be applied as soon as practicable and the dedicated register should be updated accordingly with a witness (who can be either medical or nursing staff). Where discussion with the patient indicates that someone other than the patient has accessed the VAD Kit, this should be escalated to the site SRN/SRM. This is not considered medication discrepancy. The [NMHS VAD Coordinator](#) should be notified and will follow it up on the next business day.
- If the patient is discharged from the ED or an inpatient ward, the tamper-evident tag must be removed on discharge in the presence of a witness (who can be either medical or nursing staff). This must be recorded in the dedicated register and documented in the patient's healthcare record.
- Refer to [section 9.3](#) for further information on storage of the VAD Kit.

9. The VAD substance

9.1 Prescription

The prescription of VAD substance will be organised by the Coordinating Practitioner directly with the SWPS in accordance with the [WA VAD Guidelines](#) and as described in the Mandatory WA VAD Approved Training.

9.2 Supply of the VAD substance at NMHS

Supply of the VAD substance for patients admitted to an NMHS site will be provided by the SWPS directly to the patient (or their Contact Person, or an agent of the patient) or the Administering Practitioner, depending on the Administration Decision.

The Coordinating Practitioner, the [NMHS VAD Coordinator](#), the [SWCNS](#) or another party can fulfil the role of the agent if required (e.g. if the patient is unwilling or unable to meet the SWPS representative).

9.2.1 Supply for Inpatient Self-administration

If the patient has made a self-administration decision, once requested the SWPS will meet with the patient, their Contact Person or an agent of the patient to arrange a date, time and location to receive the VAD kit.

NMHS will provide support to a patient wishing to be supplied with the VAD substance via the [NMHS VAD Coordinator](#). Once the request to supply the VAD substance has been received, the SWPS will contact the [NMHS VAD Coordinator](#) who will:

- provide SWPS with contact details of the Area Clinical Pharmacist;
- inform the patient's treating team and ward SRN/SRM about the request;
- meet with the patient, their Contact Person or an agent of the patient to arrange a date, time and location to receive the VAD Kit;
- inform the patient's treating team and the Area Clinical Pharmacist of the arranged date, time and location;
- confirm with SWPS the delivery date, time and location;
- prior to delivery of the VAD Kit to the patient, their Contact Person or an agent of the patient, the [NMHS VAD Coordinator](#) or their delegate will meet with the SWPS representative, accompany them to the ward and introduce the representative to the patient, their Contact Person or an agent of the patient, and their treating team.

A 'Do Not Disturb' sign should be placed on the door of the room while the SWPS representative provides the patient, their Contact Person or the agent of the patient with information and education about the VAD substance and adjunct medications.

Following the delivery of the VAD kit, the SWPS representative will contact the [NMHS VAD Coordinator](#) to brief them of the delivery. The [NMHS VAD Coordinator](#) will liaise with the ward SRN/SRM and the treating team to update them and coordinate appropriate VAD related care for the patient. The VAD Kit must have a tamper-evident tag applied immediately by the patient's nurse after delivery by the SWPS representative. The number of the tamper-evident tag must be recorded in a dedicated register by the nurse with a witness (who can be either medical or nursing staff).

[NMHS VAD Coordinator](#) or the site SRN/SRM can be contacted to arrange the delivery of the dedicated register.

9.2.2 Supply for Practitioner Administration

If the patient requires practitioner administration, the Administering Practitioner will arrange with the SWPS to have the VAD substance dispensed and supplied to them. Supply of the VAD substance directly to the Administering Practitioner can occur on any of the NMHS sites.

9.3 Storage of the VAD substance at NMHS

Storage of the VAD substance at NMHS will depend on the Administration Decision. Consideration should be given for the need to develop site specific procedures to reflect the context of the service.

9.3.1 Inpatient Self-administration

The VAD Kit (VAD substance and adjunct medications) must be stored in the locked box provided by the SWPS. The VAD will be locked with a PIN-coded padlock, to which **only** the patient and possibly their Contact Person have the code. The code should not be shared with hospital staff or recorded in the patient's healthcare record. On admission to a NMHS site, a tamper-evident tag should be applied to the locked box immediately, with the tag number recorded in a dedicated register by the nurse with a witness (who can be either medical or nursing staff).

- A visual inspection of the tag should be conducted at least once every shift, or at staff changeover, to ensure the tag is secure and intact. The tag number should be documented in a dedicated register (see **Figure 2**) by the nurse with a witness (who can be either medical or nursing staff). [NMHS VAD Coordinator](#) or the site SRN / SRM can be contacted to arrange the delivery of the dedicated register. **Figure 3** provides examples of entries in the dedicated register.
- The VAD Kit must remain with the patient at all times and be secured to the patient's bed where practicable. Lockable cabinets (**Figure 4**) are available for use at each NMHS site to facilitate secure storage of the VAD Kit at the patient's bedside, however the use of these lockable cabinets is not mandatory. The [NMHS VAD Coordinator](#) or the site SRN / SRM can be contacted to arrange the access to the lockable cabinet or a suitable lock for attaching to the patient's bed. One key to the cabinet should be left with the patient and the other key attached to the controlled drug keys for the duration of the patient's stay at the NMHS site.

- The dedicated register should be kept with the lockable cabinet (if used) or the VAD Kit **at all times** and returned to the hospital pharmacy at the end of the patient’s admission for archiving.

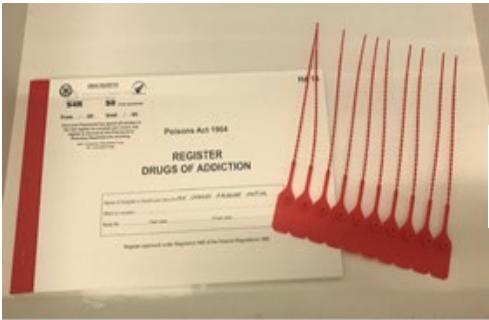


Figure 2. Dedicated register and tamper-evident tags

0001

RECORD OF DRUGS: RECEIVED, ADMINISTERED OR SUPPLIED											
DRUG (Including strength and form)			Balance					brought forward from page			
Date	Received		Identifying Details	Administered or Supplied				Balance	Name of prescriber	Administered, supplied or received by (sign & print name)	Witnessed by (sign & print name)
	Amount	Requisition number	Full name of patient or supplier	Time	Dose administered	Amount discarded	Amount issued from stock				
1/7/21	Tag No.	A123456	Patient NAME	1400	Tamper-proof tag applied			-	-	ABEPLAS Meg PLASTER	MAIRE CONNOLLY
1/7/21	Tag	A123456	Patient NAME	2200	Tamper-proof tag intact			-	-	ABEPLAS Meg PLASTER	MAIRE CONNOLLY
2/7/21	Tag	A123456	Patient NAME	0730	Tamper-proof tag intact			-	-	ABEPLAS Meg PLASTER	MAIRE CONNOLLY
2/7/21	Tag	A123456	Patient NAME	1030	Tag broken.			-	-	ABEPLAS Meg PLASTER	MAIRE CONNOLLY
2/7/21	Tag	A123457	Patient NAME	1030	New tamper-proof tag applied					ABEPLAS Meg PLASTER	MAIRE CONNOLLY

Figure 3. Examples of entries in the dedicated register (including entry where tag is found broken)



Figure 4. Lockable cabinet for the VAD Box

9.3.2 Practitioner administration

The Administering Practitioner is responsible for the safe storage of the VAD substance as outlined in the Mandatory WA VAD Approved Training and the Act.

9.4 Consideration of Staffing for Administering of the VAD substance

Care should be taken to facilitate adequate staffing to ensure good end-of-life care is provided to the patient and their families, in keeping with good clinical practice and available resources. Follow site specific procedures for further guidance.

Staff have the right to conscientiously object to participate in any or all aspects of the VAD process, or to determine what aspects of care they are comfortable providing for a patient going through the VAD process. The [NMHS VAD Coordinator](#) can be contacted for assistance and support if required.

9.5 Administering the VAD substance

The date and time of VAD administration will be discussed and planned between the patient, the Administering Practitioner, and the NMHS VAD Coordinator. Once the plan for administration is coordinated and confirmed, this should be documented in the patient's healthcare record if this is to occur during an inpatient admission and must involve the NMHS VAD Coordinator

The following personnel must be notified by the NMHS VAD Coordinator of the planned inpatient administration of the VAD substance as soon as practicable:

- The consultant from the patient's treating team
- The SRN/SRM of the admitting ward
- The shift coordinator of the admitting ward
- The divisional Co-Directors

9.5.1 Inpatient Self-administration

When a patient elects to self-administer the VAD substance on an NMHS site, the following process must be followed:

- The patient's nurse must ensure the following personnel are notified of the patient's intention to use their VAD substance:
 - the [NMHS VAD Coordinator](#), if not already aware (or the After Hours CNS if outside of business hours)
 - the consultant from the patient's treating team;
 - the SRN/SRM and shift coordinator of the admitting ward
- The ward SRN/SRM (or After Hours CNS if outside of business hours) must ensure that all staff are provided opportunities to discuss with them their individual needs and are able to decide whether or not they wish to participate in any aspect of patient care during this time.

The patient must prepare the VAD substance and administer it themselves without assistance from anyone, including staff or family. Staff may only help the patient administer other medications, such as the adjunct medications provided as part of the VAD Kit supplied by SWPS.

9.5.2 Practitioner Administration

The identified Administering Practitioner may be either an eligible doctor or an eligible nurse practitioner whom has completed the approved VAD Practitioner training and accepted the role of Administering Practitioner.

Planning between the Administering Practitioner and the patient for administration while a person is an inpatient at NMHS must involve the [NMHS VAD Coordinator](#).

When planning for practitioner administration, the Administering Practitioner should discuss with the patient the requirement for an eligible witness to be present at the time of administration. It is important that the patient is comfortable with the witness and that the witness is prepared to take on the role. The patient may have selected several carers, family or friends to be with them at the end of their life. If willing, one of these people may act as the witness to the practitioner administration of the VAD substance. NMHS recommends that NMHS staff do **not** act as witnesses for practitioner administration, however, NMHS staff *may* act as witnesses if they choose to do so.

An eligible witness:

- must have reached 18 years of age; and
- must not be an ineligible witness (i.e. they must not be a family member of the Administering Practitioner and must not be employed or engaged under a contract for services by the Administering Practitioner).

It is important to remember that by acting as a witness, the witness is signing a legal document and may be called upon to give evidence in a court or tribunal at a later date if the voluntariness of the patient is called into question. This should be considered by NMHS staff when deciding whether they are willing to be a witness.

Once the Administering Practitioner and the witness are present, it is recommended that the Administering Practitioner confirms with the patient that they are requesting the Administering Practitioner to administer the VAD substance.

The Administering Practitioner may administer the VAD substance via a PEG or nasogastric tube, or via intravenous (IV) access (peripheral or central). If the Administering Practitioner or another member of the patient's treating team is unable to successfully perform peripheral IV cannulation (in the absence of central access) they should contact the Duty Anaesthetist via site switchboard. **It is important to discuss the intended use of the IV cannula at the time of referral to ensure the anaesthetist assisting does not have a conscientious objection to involvement.**

The Administering Practitioner must administer the VAD substance - this cannot be delegated to anyone else.

9.6 Disposal of the VAD substance at NMHS

The disposal process will differ depending on the Administration Decision.

9.6.1 Inpatient Self-administration

The legislation requires the Contact Person to return any unused VAD substance within **14 days** of the patient's death to an Authorised Disposer. A list of Authorised Disposers is available on the [DoH VAD website](#).

Where the patient has died before taking the VAD substance or ingesting only part of the VAD substance, the following process will apply:

a) If the Contact Person is present at this time:

The contact person will return the substance to the kit, lock it and be responsible for returning the kit to an authorised disposer within the required time frame

b) If the Contact Person is not present at this time:

- Unused / residue VAD substance should be returned to the locked box (if not already) in the presence of a witness, the padlock reapplied and locked, and a new tamper-evident tag applied if the previous one is no longer intact. The box should be moved to a secure

location where it can be tethered to a fixture in a secure area until collected by the Contact Person;

- The shift coordinator or SRN/SRM should arrange collection of the VAD substance by the patient's Contact Person;
- Visual inspection of the tamper-evident tag should be conducted once each shift or at staff changeover to ensure it is secure and intact, and the tag number is recorded in the dedicated register with a witness (who can be either medical or nursing staff) until collection by the Contact Person;
- The Contact Person may take the locked box containing the VAD substance to a NMHS Pharmacy or an external Authorised Disposer (including SWPS), for disposal;
- Staff must record in the dedicated register and document in the patient's healthcare record that the VAD substance has been given to the Contact Person (including the full name of the Contact Person) for transfer to an Authorised Disposer.
- The dedicated register must be returned to the site pharmacy for archiving by the Clinical Pharmacist.

9.6.2 Practitioner Administration

The Administering Practitioner must dispose of any unused or remaining VAD substance as per the instructions provided by SWPS, and complete the *Practitioner Disposal Form* and submit to the VAD Board via [VAD-IMS](#) within two (2) business days of disposal.

10 Other Scenarios

10.1 Patient with access to VAD substance is receiving Hospital in the Home (HITH) care

If a NMHS staff member is visiting a patient in the community who has been deemed eligible for VAD, staff should continue to provide all aspects of non-VAD related care to the patient.

If the patient is known to have access to the VAD substance, and is found unconscious or without signs of life, the patient should **not be resuscitated** unless there is a strong indication otherwise. The Act protects staff who act in good faith in this circumstance.

Staff must:

- notify their line manager, and/or the treating Consultant immediately;
- consider providing the patient with medications to treat any symptoms as appropriate;
- not give medications that will hasten death.

10.2 Patient brought into ED with decreased GCS or is unresponsive and is known to have access to the VAD substance

If a patient is known to have access to the VAD substance, the patient should **not be resuscitated** unless there is a strong indication otherwise. **The Act protects health professionals who act in good faith in this regard.**

Staff must:

- notify ED CNS and Duty Consultant immediately;
- consider providing the patient with medications to treat any symptoms as appropriate;
- not give medications that will hasten death;
- notify the [NMHS VAD Coordinator](#).

11 Care of the Patient between VAD Administration and Death

For **oral administration (or via NGT or PEG tube)**, it is expected that most patients die within 30 to 60 minutes, however times can vary. Patients typically lose consciousness rapidly post ingestion.

For **IV administration**, most patients lose consciousness very rapidly and die in under 10 minutes.

Usual end-of-life care should occur with appropriate support for the patient, their family and staff in keeping with good clinical practice.

Non-VAD substance medications aimed at symptom relief and unexpected events should be administered as required, but under no circumstances should medication be given with the sole intent of hastening death.

12 Death Certification

The Act requires there **must not** be any reference to VAD on the patient's death certificate. **To do so is a breach of the Act.** The medical practitioner who confirms and certifies the patient's death, will record their underlying illness or medical condition as the cause of death.

A VAD death does not have to be reported to the Coroner as a matter of course, other than where:

- the person was, immediately before their death, 'held in care' (as defined under the *Coroner's Act 1996*); or
- where the death was not in accordance with, or suspected not to be in accordance with, the Act; or
- the death meets any of the criteria listed on the [Death in Hospital](#) form.

When the manner of the patient's death is by VAD, the deceased person's body may be released to a funeral director, Single Funeral Permit holder or family in accordance with patient's wishes and / or instruction of their family / carer or transferred to the State Mortuary. Where the body is released directly to the family, staff are encouraged to provide families with information relating to public health and legislative responsibilities (see [Department of Health Guideline for the preparation of the deceased for burial or cremation](#)).

13 Discussing an Assisted Death

Health professionals should note that the Act includes protections and offences that aim to further protect the privacy of patients and their families. It is important to be aware that the Act **does not** allow a person to disclose information they have obtained as a result of performing a function under the Act (unless they meet special criteria to do so). Information about patients who access VAD should **not** be shared, and anyone who discloses that the person has died because they accessed VAD is potentially committing an offence under the Act. Refer to [NMHS VAD Policy](#) Section 13 - Legal Context of VAD for Practitioners.

14 Documentation

For patients who have not accessed NMHS previously and are admitted for administration of the VAD substance, a healthcare record must be created, but documentation may be similar to that required for a same-day admission.

14.1 Documentation of Discussions about VAD

Health professionals must ensure they document any conversations with the patient, their family or carer in relation to VAD in the patient's healthcare record. As a guide, the following should be documented:

- the context of the conversation and who has initiated the discussion;
- the exact phrases used by the patient, where possible;
- the exact phrases of the health professional's response, where possible; and
- a summary of the conversation, actions of the health professional including any escalation responses or referrals.

All documentation in patient's healthcare records in relation to VAD must comply with [NMHS Clinical Documentation Policy](#). VAD stickers (see **Figure 5**) are available in clinical areas to indicate when a VAD assessment or interaction by a Participating Practitioner has occurred in a NMHS hospital site.

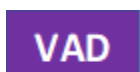


Figure 5 VAD sticker

Entries in uncontrolled documents (eg handover sheets, discharge summaries) should not mention VAD unless specific patient consent has been sought. Doing so could place a practitioner at risk of breaching s106 of the Act. A generally acceptable alternative is 'end-of-life care'.

14.2 Documentation of the First Request

Where a patient has made a First Request to a medical practitioner, Section 21 of the Act requires the following documentation be entered in the patient's healthcare record:

- a First Request has been made by the patient;
- the decision by the medical practitioner to accept or refuse;
- if refused, reason for refusal, which may be one of the following:
 - i. Conscientious objection
 - ii. Ineligible to accept
 - iii. Unwilling to accept
 - iv. Unable to accept
- if they have provided the patient with the approved information.

A hospital Medical Record form (VAD – First Request Form) is available at all NMHS sites to facilitate documentation of a First Request in the patient's healthcare record. Note that a First Request form must also be completed on VAD-IMS as per Box 1 (page 11).

14.3 Documentation of Remaining Steps of the VAD Process, including Referrals

Participating VAD practitioners must complete all mandated documentation in relating to VAD as outlined in the Act. This documentation is stored in a separate, secure portal referred to as [VAD-IMS](#), which is only accessible by trained VAD participating practitioners and the VAD Board Secretariat. The documentation requirements are explained in details in the mandatory WA VAD Approved Training and the [WA VAD Guidelines](#).

A brief entry should be made using the 'VAD' sticker (**Figure 5**) in the patient's healthcare record of any formal assessment, referral or interaction conducted at a NMHS site by a VAD participating practitioner. Noting that the final determination of eligibility is documented securely in VAD-IMS, it is acceptable to document clinical information in the medical record where this describes information directly related to the determination of eligibility. **Figure 6** provides an example of a brief entry.

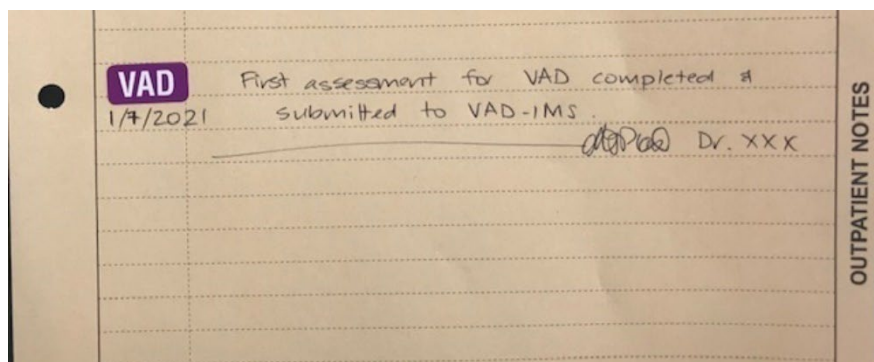


Figure 6 Example of brief entry in the patient's healthcare record

15 Support Resources

15.1 Support Resources for Families, Carers and Friends

Medical practitioners and other health professionals should provide bereavement support to a patient's family, carers and friends in the same way that they usually would, whether directly or via existing site bereavement support services. Where palliative care services have been involved in the care of the patient, they will generally be able to offer bereavement support, or referral to other services.

Planning for death should be an ongoing conversation with the patient and, if they choose, their carers, family or friends. The plan should include practical information such as who should be contacted once the patient has died, who will remove the patient's body from the place of death, any relevant location-based procedures and local bereavement services that are available to provide continued support.

The [NMHS VAD Coordinator](#) or the [SWCNS](#) can be contacted to provide support. Referrals to Social Work and Chaplain Services within NMHS should be considered when appropriate. Refer to [Appendix 2](#) or a list of resources which may be useful in helping the patient's carers, family and friends as part of bereavement support.

Additional information for carers, family and friends can be found in the [DoH Supporting someone through the process](#) and [DoH Considerations for an assisted death](#) information sheets.

15.2 Support for Staff

NMHS acknowledges that staff may be personally affected by VAD in different ways. The [Employee Assistance Program \(EAP\)](#) can be accessed for support. Additional support can be provided by the [NMHS VAD Coordinator](#).

Below are additional support resources:

For Medical Practitioners

- [Department of Health \(WA\) Voluntary Assisted Dying website](#)
- [RACGP Support Program](#)
- [Royal Australasian College of Physicians \(RACP\) Resources](#)

- [Royal Australian and New Zealand College of Psychiatrists \(RANZCP\) Wellbeing Support](#)
- [The Doctors' Health Advisory Service Western Australia \(DHASWA\)](#)
- [DRS4DRS](#)
- [Australasian College for Emergency Medicine](#)
- [Australian and New Zealand College of Anaesthetists and Faculty of Pain Medicine \(ANZCA\)](#)

For Nurse Practitioners

- [Department of Health \(WA\) Voluntary Assisted Dying website](#)
- Peer support from the Australian College of Nurse Practitioners (ACNP) (Email: admin@acnp.org.au)
- Australian College of Nursing (ACN)
- [Nursing and Midwifery Board of Australia \(NMBA\) Nurse and Midwife Support](#)
- Australian Primary Care Nurses Association (APNA) Support (Email: nursesupport@apna.asn.au)

See [Appendix 2](#) for additional general resources for mental health and wellbeing.

16 Definitions

Administering practitioner	The medical practitioner or nurse practitioner who administers the VAD substance to a patient. If the patient chooses practitioner administration, the Coordinating Practitioner can be the Administering Practitioner.
Authorised disposer	A person who is authorised under subsection 3 of the Act to dispose of the prescribed VAD substance.
An agent of the patient	An agent of the patient is someone who acts on the patient's behalf to: • collect the VAD substance from the SWPS; • courier the VAD substance to the patient; and • provide the patient with all the information given by the SWPS on the VAD substance.
Carriage Service	Communication aids including phone, fax, email, videoconferencing (including Telehealth), internet and the like.
Conscientious Objector	A person who has considered VAD and whose beliefs and values conflict with VAD and do not wish to participate in any aspect of VAD process.
Conscientious Provider	A person who has considered how VAD aligns with their beliefs and values and is willing to participate in the process. The level of participation may vary from person to person and may change and vary with time.
Consulting Practitioner	A medical practitioner who accepts a referral to conduct a consulting assessment of the patient.
Contact Person	A person appointed by a patient (under section 65(1) of the Act) who has made a self-administration decision to carry out specific activities required by the Act. Contact Person has the responsibility of returning any unused or remaining VAD substance to the WA VAD Statewide Pharmacy Service. Other specific roles are explained further in the Being the Contact Person information sheet .

Coordinating Practitioner	The medical practitioner who accepts the patient's First Request or the Consulting Practitioner who accepts the transfer of the role of the Coordinating Practitioner.
Eligible medical practitioner	A medical practitioner who meets the eligibility criteria listed under section 17 of the Act and who has successfully completed the WA VAD Approved Training. Eligibility criteria can be found in the WA VAD Guidelines, section 4.2
Eligible nurse practitioner	A nurse practitioner who meets the eligibility criteria listed under section 54 of the Act and who has successfully completed the WA VAD Approved Training. Eligibility criteria can be found in the WA VAD Guidelines, section 4.2
First Request	A clear and unambiguous request for access to VAD made to a medical practitioner during a medical consultation under section 18(1) of the Act. This is the first of three requests a patient must make as part of the process to access VAD.
Final Request	The final request for access VAD that a patient makes to the Coordinating Practitioner after completing the Written Declaration. This is the last of three requests a patient must make to access VAD.
Medical Consultation	An appointment or meeting with a medical practitioner to seek medical advice or treatment.
Nurse	In this document, the term 'nurse' encompasses registered nurses and midwives.
Nurse Practitioner	A person registered in the nursing profession (other than as a student) whose registration is endorsed as nurse practitioner. A nurse practitioner has an additional masters degree and is licensed to work at an advanced practice level.
Palliative Care	An approach to care that improves the quality of life of people and their families who are facing the problems associated with a progressive illness. It aims to neither prolong nor hasten death. Palliative care prevents and relieves suffering through early identification and assessment, treats pain and other physical, psychosocial, and spiritual problems and addresses practical issues.
Self administration	The process whereby a person, who has been assessed as eligible for VAD and made a self-administration decision, prepares and ingests the VAD substance themselves.
Statewide Care Navigator Service (SWCNS)	The service established to provide a statewide point of contact for information and assistance relating to VAD in Western Australia. This includes help locating a practitioner who is willing and eligible to participate and providing support to staff and patients.
Statewide Pharmacy Service (SWPS)	The statewide pharmacy service established to ensure the VAD substance is provided in a safe, equitable, and patient-centred manner.
Voluntary Assisted Dying (VAD)	The legal process that enables an eligible person to access, administer or have administered to them, the VAD substance for the purpose of causing their death.
VAD Box	The box provided to securely storing the VAD Kit and lockable using a code padlock.
VAD Kit	Refers to the VAD substance and adjunct medications supplied by the SWPS.

VAD Practitioner	Medical Practitioners and Nurse Practitioners who meet the eligibility requirements, and who have successfully completed the WA VAD Approved Training to participate in the voluntary assisted dying process as a VAD Practitioner
VAD Substance	A controlled substance for the purpose of causing a person's death, referred to as the 'VAD substance' in the Act.
Written Declaration	The Written Declaration is a formal written request for access to VAD. This is the second of three requests a person must make as part of the process to access VAD.
SRN	Senior Registered Nurse
SRM	Senior Registered Midwife

17 Compliance and Evaluation

Each NMHS site or service Executive Director is to ensure local compliance with this Procedure.

18 Document Control

Version	Amendment	TRIM Ref	Published Date
1.0	Original version	D/21/53957	30 June 2021
2.0	Scheduled 3-month post implementation review with amendment	D/21/98352	17/01/2022
3.0	Scheduled 12-month post implementation review with major amendments made. A new standard 3-yearly policy review cycle will commence. Changes include: • expansion and clarification of definitions • clarification of information regarding use of language services; • clarification of medical staff obligations under the Act; • clarification of the admission referrals and pathways for inpatient administration of VAD Substance; • clarification of requirements for supply of the VAD substance.	D/23/142226	30/10/2023 (NET approved 20/10/2023)

Related policies, procedures and guidelines

WA Health Policy Framework/Mandatory Policies

[Credentialing and Defining Scope of Clinical Practice Policy \(MP 0084/18\)](#)

[High Risk Medication Policy \(MP 0131/20\)](#)

[Language Services Policy \(MP 0051/17\) and related documents](#)

[Language Services Guidelines](#)

[Managing Voluntary Assisted Dying Policy \(MP 0154/21\)](#)

[Medicines Handling Policy \(MP 139/20\)](#)

[Western Australian Voluntary Assisted Dying Guidelines](#)

NMHS Policy Documents

[NMHS Carers Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS Destruction, Discard and Disposal of S8 and S4 Medicines Policy](#) and [Procedure](#)
[NMHS Discrepancy Management of S8 and S4R Medicines Policy](#)
[NMHS Healthcare Record – Access, Use and Disclosure Policy](#)
[NMHS Healthcare Record Management Policy](#)
[NMHS Management of Psychosocial Hazards in the Workplace Guideline](#)
[NMHS Mandatory Training Policy](#)
[NMHS My Health Record \(MHR\) System Control, Administration, Access, Upload and Security Procedure](#)
[NMHS Voluntary Assisted Dying Policy](#)
[NMHS Witnessing of Patient Wills and Other Legal Documents Policy](#)
[WNHS Policy Documents](#)
[WNHS Procedure Language Services](#)

References

[Coroners Act 1996](#)
[Criminal Code Act 1995 \(Commonwealth\)](#)
[Health and Disability Services \(Complaints\) Act 1995](#)
[Health Practitioner Regulation National Law \(Western Australia\) Act 2010](#)
[Medicines and Poisons Act 2014](#)
[Mental Health Act 2014](#)
[Voluntary Assisted Dying Act 2019](#)

Resources

[Employee Assistance Program \(EAP\)](#)
[Voluntary Assisted Dying - Information Management System \(VAD-IMS\)](#)
[WA Health Voluntary Assisted Dying resources for health professionals](#)
[WA VAD Statewide Care Navigator Service \(SWCNS\)](#)

Acknowledgement

Some content in this Procedure is based on the resources developed by the Austin Health and East Metropolitan Health Service. This Procedure is also informed by the *Voluntary Assistant Dying Act 2019 (WA)*, the Managing Voluntary Assisted Dying Policy (MP 0154/21) and the Western Australian Voluntary Assisted Dying Guidelines developed by the Western Australia Department of Health.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1. VAD Process – Role Eligibility and Responsibilities

Eligibility / Responsibility	Consultant and PGY10+	Medical Staff (including Registrars)	Nurse Practitioners	All other clinical staff
Eligible to complete the WA VAD Approved Training	Yes - once completed the Department of Health identity and eligibility verification process	No	Yes - once completed the Department of Health identity and eligibility verification process	No
Provide information to a patient about VAD	Yes - only if the patient raises it first or as part of a wider discussion about the patient's treatment and end-of-life priorities	Yes - only if the patient raises it first and they are comfortable to do so	Yes - only if the patient raises it first or as part of a wider discussion about the patient's treatment and end-of-life priorities	Yes - only if the patient raises it first and they are comfortable to do so
Respond to a First Request	Yes - All medical practitioners must respond and complete the 4 steps outlined in section 5.1.3 of the NMHS VAD Procedure	Yes, but can't accept the request - All medical practitioners must respond and complete the 4 steps outlined in section 5.1.3 of the NMHS VAD Procedure	Yes, but can't accept the request – The patient should be provided with the <i>Approved information for a person making a First Request</i> and the VAD Coordinator (with the patient's consent) should be informed	Yes, but can't accept the request – The patient should be provided with the <i>Approved information for a person making a First Request</i> and the VAD Coordinator (with the patient's consent) should be informed
Accept a First Request (i.e. become the patient's Coordinating Practitioner)	Yes (if they choose to do so) - the request can be accepted prior to commencing the WA VAD Approved Training providing the Consultant is willing to complete the training	No – Most junior medical staff will not meet the eligibility criteria to complete the WA VAD Approved Training	No - Only a medical practitioner can accept a first request and become the patient's Coordinating Practitioner	No - Only a medical practitioner can accept a first request and become the patient's Coordinating Practitioner
Complete the First Assessment	No – unless they are the patient's Coordinating Practitioner	No	No	No
Complete the Consulting Assessment	Yes (if they choose to do so) – but only if they are NOT the patient's Coordinating Practitioner and have completed (or are willing to complete) the WA VAD Approved Training.	No	No	No

Eligibility / Responsibility	Consultant and PGY10+	Medical Staff (including Registrars)	Nurse Practitioners	All other clinical staff
Accept a referral to assist with determining the patient's eligibility criteria	Yes - If they have the appropriate skills and training to provide a determination in relation to the patient's diagnosis, prognosis or decision-making capacity	No	No	No – With the possible exception of a Social Worker confirming the patient's citizenship or residency status
Witness a Written Declaration	No – unless they understand the implications described in section 5.4 of the NMHS VAD Procedure and are not the patient's Coordinating Practitioner	No – unless they understand the implications described in section 5.4 of the NMHS VAD Procedure	No – unless they understand the implications described in section 5.4 of the NMHS VAD Procedure	No – unless they understand the implications described in section 5.4 of the NMHS VAD Procedure
Accept and Document a Final Request	No - unless they are the patient's Coordinating Practitioner	No	No	No
Conduct and Document the Final Review and Administration Decision	No - unless they are the patient's Coordinating Practitioner	No	No	No
Be appointed as the patient's *Contact Person	No – unless the patient is a family member or friend	No – unless the patient is a family member or friend	No – unless the patient is a family member or friend	No – unless the patient is a family member or friend
Handle the substance	No - unless they are acting as an *Agent for the patient or they are the patient's Administering Practitioner	No - unless they are acting as an *Agent for the patient	No - unless they are acting as an *Agent for the patient or they are the patient's Administering Practitioner	No - unless they are acting as an *Agent for the patient
Complete the Medical Certificate of Cause of Death	Yes – noting they must not include any reference to VAD	Yes – noting they must not include any reference to VAD	No	No

**Refer to [section 16](#) of the NMHS VAD Procedure for definition*

Appendix 2. Support Resources for Families, Carers and Friends

- **The Statewide Care Navigator Service (SWCNS)**
The SWCNS can be contacted by email and phone during standard work hours (8:30am-5pm, Monday to Friday excluding Public Holidays):
 - Email: VADcarenavigator@health.wa.gov.au
 - Phone: (08) 9431 2755
- **Department of Health (WA) Voluntary Assisted Dying** website
<https://ww2.health.wa.gov.au/voluntaryassisteddying>
- **Australian Centre for Grief and Bereavement** helps family, friends, carers and deal with the death of a loved one and put them in touch with appropriate support groups.
Telephone: 1800 642 066, Monday to Friday (9:00 am – 5:00 pm) or access the website www.grief.org.au
- **Palliative Care WA** provides the WA community with a palliative care information and support line which can be used to access support for dealing with grief and loss.
Telephone: 1800 573 299, Monday to Sunday (9:00 am – 4:30 pm) or access the website www.palliativecarewa.asn.au/information-and-support
- **Carers WA** can provide counselling support for carers who are going through the experience of grief and loss.
Telephone: 1800 007 332, Monday to Friday (8:30 am – 4:30 pm) or access the website www.carerswa.asn.au
- **Lifeline** can provide crisis support to anyone who is need of immediate help to deal with emotional distress.
Telephone: 13 11 14, (any time day or night) or access the website www.lifeline.org.au
- **13YARN** offers 24/7 crisis support provided by Aboriginal and Torres Strait Islander crisis supporters.
Telephone: 13 92 76 or visit the website: www.13yarn.org.au
- **Beyond Blue** provides information and support to help individuals experiencing anxiety and depression.
Telephone: 1300 224 636 or visit the website: <https://www.beyondblue.org.au/>
- **TEN (The Essential Network for Health Professionals)** - Block Dog Institute provides a range of resources for health professionals, including mental health self-assessment, confidential online courses, resources for managing burnout, and professional peer support and mentoring.
Visit website: <https://www.blackdoginstitute.org.au/the-essential-network/>